



Sex: Female
D.O.B.:
MRN #:
Ref Phy

SPECIMEN INFO

Collected:
Received:
Reported:

SURGICAL PATHOLOGY REPORT

DIAGNOSIS

DIAGNOSIS:

- A. Ureter, right, biopsy:
No neoplastic process identified.
- B. Ureter, left, biopsy:
No neoplastic process identified.
- C. Urinary bladder, uterus, cervix, right ovary, cystectomy with hysterectomy and right salpingo-oophorectomy:
Poorly differentiated invasive urothelial carcinoma.

*ICD-O-3
Carcinoma, urothelial NOS
8120/3
Site Bladder NOS
C67.9
JTD 6/12/13*

Tumor Characteristics:

1. Histologic type: Urothelial (transitional) carcinoma.
2. Histologic grade: Poorly differentiated (G3).
3. Tumor size: 7.5 cm. in greatest dimension.
4. Microscopic tumor extension: Tumor shows invasion of the muscularis propria with full thickness of the bladder wall involvement; no diagnostic tumor identified within the perivesicle adipose tissue.
5. Lymphovascular space invasion: Not identified.

Surgical Margins:

Tumor not identified within sections of the right ureter (specimen A), left ureter (specimen B), urethra or inked peritoneal surface.

Other:

1. Extensive acute and chronic inflammation with areas of foreign body giant cell reaction within the perivesicle fibroadipose tissue.

Uterus-

Endometrium- Atrophic pattern; benign endometrial polyp.
Uterine cervix- Benign mucinous cysts with no neoplastic process identified.
Myometrium- Leiomyomas.
Uterine serosa- Serosal fibrous adhesions.

Right ovary-

Multiple serous cysts.
Serosal fibrous adhesions.

Right fallopian tube-

Benign fallopian tube with paratubal cysts.

Lymph node-

No neoplastic process identified in one lymph node.
See specimen D for results of additional separately submitted lymph nodes labeled pelvic lymph nodes.

AJCC cancer staging for cancer of the urinary bladder- pTNM stage: T2bN0.

D. Lymph nodes, pelvic, excision:

No metastatic neoplasm identified in seven lymph nodes.

Electronic Signature:

COMMENTS:

Histologic sections confirm the presence of an invasive poorly differentiated urothelial carcinoma. The neoplasm shows full thickness invasion of the bladder wall including full thickness of the muscularis propria. The tumor does not definitely involve the perivesicle fibroadipose tissue. There is extensive acute and chronic inflammation and foreign body giant cell reaction in areas of the perivesicle fibroadipose tissue. Given that tumor is not clearly identified microscopically within the perivesicle tissue, the neoplasm is assigned an AJCC pathologic staging of T2b. This has been reviewed with Dr [redacted] is in agreement.

CLINICAL INFORMATION

CLINICAL HISTORY:

Preoperative Diagnosis: Bladder carcinoma

Postoperative Diagnosis:

Symptoms/Radiologic Findings:

SPECIMENS:

A. Right ureter, clip on distal end with frozen section

B. Left ureter, with frozen section

C. Bladder, uterus, cervix, right ovary

D. Pelvic lymph nodes

SPECIMEN DATA

GROSS DESCRIPTION:

The specimen is received in four containers labeled with the patient's name

A. Container A is received fresh for frozen section additionally labeled 'right ureter, clip at distal end' and contains a 0.5 cm in length x 0.5 cm in diameter cylindrical portion of yellow-tan soft tissue bearing a clip at the distal end. A representative section is submitted for frozen section with residual entirely resubmitted for permanent section in cassette A1 label

B. Container B is received fresh for frozen section additionally labeled 'left ureter' and contains a 0.6 cm in length x 0.7 cm in diameter yellow-tan cylindrical soft tissue, which also bears a clip at one end. Representative section is submitted for frozen section with the residual entirely resubmitted for permanent section in cassette B1 label

C. Container C is received with formalin additionally labeled 'uterus, cervix, right ovary and bladder' and contains an anterior exenteration specimen comprised of bladder (8.0 x 7.0 x 6.0 cm), urethra (3.0 cm in length x 1.0 cm in diameter), anterior vaginal wall (6.5 x 4.5 cm), right ureter (5.0 cm in length x 0.4 cm in diameter), left ureter (4.5 cm in length x 0.5 cm), uterine cervix (2.0 cm in length x 4.3 x 3.2 cm), uterine corpus (4.5 cm in length x 4.0 x 2.5 cm), right adnexal complex (4.5 x 3.0 x 2.0 cm), and attached perivesicular fat. A left uterine adnexal complex is not present. The soft tissue margins are inked. The bladder is opened to reveal a 7.5 x 6.5 x 1.5 cm yellow-tan fungating and friable mass that involves the anterior, right and left lateral bladder walls, as well as the bladder dome. This mass approaches to within 0.1 cm of the right ureteral orifice, 1.5 cm of the left ureteral orifice and 7.0 cm from the urethral margin. Normal appearing pink-tan focally erythematous but glistening bladder mucosa is present near the trigone. On section, this mass extends into the detrusor muscle and approaches to within 0.1 cm of the inked deep margin, as well as to within 0.1 cm of the inked deep peritonealized surface. This mass does not involve the uterine wall. The endocervical canal is yellow-tan cystic and mucoid with the usual folds. The triangular endometrium is pink-tan and glistening with an average thickness of 0.1 cm. The posterior aspect is remarkable for 1.0 x 1.0 x 0.5 cm pink-tan polypoid lesion. On section, no areas of invasion are identified. The surrounding myometrium is pink-tan and fibrous with an average thickness of 1.0 cm. Within the myometrium on the posterior aspect is a 0.7 cm well delineated intramural nodule which features a yellow-tan diffusely calcified cut surface. Sections through the right adnexal complex reveals a 3.5 cm in length x 0.5 cm in diameter segment of fimbriated fallopian tube. The yellow-tan lobulated right ovary is 2.5 x 2.0 x 1.5 cm. Sectioning reveals a yellow-tan fibrous cut surface with a central multiloculated, cystic corpus albicans.

On section and palpation, two firm fatty possible lymph nodes are identified within the associated fat. They are 0.2 and 0.4 cm in greatest dimension.

Representative sections are submitted in cassettes C1 through 19 labeled

signed as follows: 1, urethral margin, en face; 2, trigone

to include vaginal wall; 3, right ureteral orifice to include mass; 4, left ureteral orifice; 5, right lateral bladder wall to include mass and inked deep margin, perpendicular; 6, left lateral bladder wall to include mass and inked deep margin, perpendicular; 7 and 8, anterior bladder wall to include mass and inked deep margin, perpendicular; 9 and 10, bladder dome to include mass and inked peritonealized surface, perpendicular; 11, dome to include mass and adjacent normal appearing bladder mucosa; 12, posterior bladder wall to include mass and anterior endoectocervix; 13, posterior endoectocervix; 14, anterior endomyometrium; 15, posterior endomyometrium to include polypoid lesion; 16, posterior endomyometrium to include remainder of polypoid lesion and calcified nodule (submitted after decalcification); 17, right fallopian tube; 18, right ovary; 19, two whole possible lymph nodes. Additionally, a yellow, green and blue cassette are submitted for genomics research each labeled

D. Container D is additionally labeled 'left and right pelvic lymph nodes' and contains a 6.0 x 4.5 x 3.0 cm aggregate of yellow-tan lobulated fibrofatty soft tissue. On palpation, seven firm fatty possible lymph nodes are identified ranging from 0.4 up to 2.3 cm in greatest dimension. They are entirely submitted in cassettes D1 through 4 labeled signed as follows: D1, three whole possible lymph nodes; D2, two whole possible bisected lymph nodes (one inked); D3 and 4, one whole possible bisected lymph node in each cassette.

INTRA-OPERATIVE CONSULTATION:

FROZEN SECTION DIAGNOSES:

Part A: No tumor identified on frozen section.

Part B: No tumor identified on frozen section.

Criteria	Yes	No
Diagnosis Discrepancy		☒
Prognosis/Tumor Site Discrepancy		☒
HPV Discrepancy		☒
Prior Malignancy History		☒
Dual/Synchronous Primary Noted		☒
Case by (circle):	QUALIFIED	DISQUALIFIED
Reviewed by: <i>Amie</i>	Date Reviewed: <i>6/10/13</i>	